



Value-based programs

Hospital-Acquired Condition Reduction Program

Hospital Readmissions Reduction Program (HRRP)

The Hospital Value-Based Purchasing (VBP) Program

Other value-based programs

Medicare Access and CHIP Reauthorization Act

Physician Value-Based Payment Modifier (VM)

Value-Based Purchasing (SNF VBP) Program

Quality Payment Program

Quality Payment Program Cost Measures

Hospital Readmissions Reduction Program

What is the Hospital Readmissions Reduction Program?

The Hospital Readmissions Reduction Program is a Medicare value-based purchasing program that, for example, encourages hospitals to improve communication and care coordination to better engage patients and caregivers in discharge plans and, in turn, reduce avoidable readmissions.

Section 1886(q) of the Social Security Act sets forth the statutory requirements for the Hospital Readmissions Reduction Program, which required the Secretary of the Department of Health and Human Services to establish the Hospital Readmissions Reduction Program starting October 1, 2012 (i.e., federal fiscal year [FY] 2013).

Why is the Hospital Readmissions Reduction Program important?

The Hospital Readmissions Reduction Program improves Americans' health care by linking payment to the quality of hospital care. CMS incentivizes hospitals, for example, to improve communication and care coordination efforts to better engage patients and caregivers on post-discharge planning.

We've included measures of conditions and procedures that make a big difference in the lives of large numbers of people with Medicare. The Hospital Readmissions Reduction Program, along with the Hospital Value Based Purchasing (VBP) and Hospital-Acquired Condition (HAC) Reduction Programs, is a major part of how we add quality measurement, transparency, and improvement to value-based payment in the inpatient care setting.

Research shows that hospital readmission rates differ across the nation. This gives us an opportunity to improve the quality of care and save taxpayer dollars by incentivizing providers to reduce excess readmissions.

What are applicable Hospital Readmissions Reduction Program hospitals?

For a full description of subsection (d) hospitals and classifications of excluded hospitals, refer to the Code of Federal Regulations ([42 CFR 412.20 and 412.23](#)).

CMS exempts Maryland hospitals from the Hospital Readmissions Reduction Program payment reductions because an agreement between CMS and Maryland.

What measures are included in the Hospital Readmissions Reduction Program?

We use the excess readmission ratio (ERR) to assess hospital performance. The ERR measures a hospital's relative performance and is a ratio of the predicted-to-expected unplanned readmissions rates. We calculate an ERR for each condition or procedure included in the program:

- Acute Myocardial Infarction (AMI)
- Chronic Obstructive Pulmonary Disease (COPD)
- Heart Failure (HF)
- Pneumonia
- Coronary Artery Bypass Graft (CABG) Surgery
- Elective Primary Total Hip Arthroplasty and/or Total Knee Arthroplasty (THA/TKA)

The Hospital Readmissions Reduction Program doesn't include the Hospital-Wide All-Cause Readmission (HWR) measure, which is included in the Hospital Inpatient Quality Reporting (IQR) Program.

What counts as a readmission under the Hospital Readmissions Reduction Program?

The Hospital Readmissions Reduction Program 30-day risk standardized unplanned readmission measures capture unplanned readmissions that happen

within 30 days of discharge from the index (that is, initial) admission. The readmission measures include patients who are readmitted to the same hospital, or another applicable acute care hospital, no matter the principal diagnosis. The measures exclude some planned readmissions.

How does the Hospital Readmissions Reduction Program adjust payments?

For each eligible hospital, we calculate the payment adjustment factor. The payment adjustment factor corresponds to the percent a hospital's payment is reduced. The payment adjustment factor is a weighted average of a hospital's performance across the readmission measures during the Hospital Readmissions Reduction Program performance period. We apply the payment adjustment factor for all discharges in the applicable fiscal year, no matter the condition.

For more information, visit the QualityNet [Methodology](#) page or download the Hospital Readmissions Reduction Program Payment Reduction Methodology Infographic on the QualityNet Resources page.

What is the Review and Correction period?

The 30-day Review and Correction period allows applicable hospitals to review and correct their Hospital Readmissions Reduction Program payment reduction and component result calculations as reflected in their Hospital Specific Report (HSR) (that is, Payment Adjustment Factor, Dual Proportion, Peer Group Assignment, Neutrality Modifier, ERR, and Peer Group Median ERRs) prior to them being used to adjust payments. Hospitals cannot submit corrections to the underlying claims data or add new claims to the data extract during this period.

Each program year, we let hospitals know the exact dates of the Review and Correction period via the Hospital Value-Based Purchasing (VBP) Listserv or on the QualityNet website.

How will I know whether CMS incorporated changes to the Hospital Readmissions Reduction Program?

Changes to the program are announced via the Hospital VBP [QualityNet listserv](#). Visit the [Sign Up for Email Updates](#) page on the QualityNet website and select the “Hospital VBP Notify” List under “Quality Reporting” to join the Hospital VBP Listserv.

Where can I find more information about the Hospital Readmissions Reduction Program?

For more detailed information, visit the [CMS Hospital Readmissions Reduction Program](#) page or [QualityNet Hospital Readmissions Reduction Program](#) page.

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